

NURSING VALUE ASSESSMENT

Deaconess - Midtown (Total MS Beds)

143 beds · 149 nurse FTEs · Inpatient Nursing

PREPARED BY

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Where Nursing Value Actually Lives

Ambient documentation creates value across four distinct buckets. For nursing, three of them carry real dollars in this model — and the fourth, revenue, is tracked separately in the physician and APP models where billing originates. We separate them because the strategic implications of each are different.

CAPACITY

Tracked

Reclaimed documentation time goes back to bedside care. We track the ratio of bedside-to-charting time post-deployment as a leading indicator that the model is working.

WORKFORCE

\$671K

Documentation burden is among the top drivers of nurse turnover. The hypothesis we model: reducing end-of-shift charting moves retention, the agency spend that follows every vacancy, and the overtime budget — three distinct labor lines that all share one root cause.

QUALITY

\$311K (potential)

Preventable harm events — HAPIs, falls, CAUTI, CLABSI — and sepsis bundle compliance are sensitive to documentation timing. The hypothesis we model: real-time flowsheet capture surfaces the visibility for earlier intervention, with each unit's actual outcome shaped by clinical practice.

REVENUE

Tracked Separately

Revenue impact from documentation is captured in the physician and APP models, where billing originates. Nursing's measurable value lives in quality outcomes and labor economics — which is what this model focuses on.

WHAT YOUR CHOICES REVEAL

Your model combines overtime reduction with retention savings. OT reduction is grounded in your current overtime spend; retention reflects the burnout-related turnover that documentation burden accelerates. Together they form a defensible staffing ROI.

Documentation burden is among the top reasons nurses leave — and stay late.

Three labor lines — turnover, agency premium, and overtime — all linked to the same upstream factor: how long it takes to finish charting at the end of a shift.

RN RETENTION

\$90K

Documentation burden is among the factors associated with burnout and turnover. Reducing burden is modeled to help retain experienced nurses who might otherwise leave.

NURSE FTES	149
ANNUAL TURNOVER RATE	18%
BURNOUT-RELATED SHARE	40%
DOCUMENTATION IMPACT ON BURNOUT TURNOVER	15%
REPLACEMENT COST PER NURSE	\$56,300
= RETENTION SAVINGS	\$90K

Source: NSI Nursing Solutions 2024 turnover benchmark.

AGENCY & TRAVEL NURSE REDUCTION

\$48K

•LINKED TO RETENTION

When nurses leave, hospitals typically fill gaps with agency labor at 2–3× the cost. Improved retention is modeled to reduce that premium-labor dependency.

WEEKS OF AGENCY COVERAGE AVOIDED PER VACANCY	12 wks
WEEKLY AGENCY PREMIUM	\$2,500
= AGENCY COST AVOIDED	\$48K

OVERTIME REDUCTION

\$533K

When nurses spend less time documenting at end of shift, OT hours decrease. The most directly measurable line in the payroll budget.

NURSE FTES	149
OT HRS PER NURSE PER WEEK	2.3 hrs
DOCUMENTATION-DRIVEN OT REDUCTION	40%
OT HOURLY RATE	\$75/hr
WEEKS PER YEAR	52
= OT SAVINGS	\$533K

WORKFORCE SUBTOTAL \$671K

A leading indicator, not a billable line.

When nurses finish charting on shift, more of the next hour can go to direct patient care rather than catch-up documentation. We track the ratio of bedside-to-charting time as a leading indicator that the model is taking hold – workforce and quality outcomes are typically downstream of that shift, though the magnitude is unit-specific.

PATIENT DAYS/YR	HRS RETURNED/YR	MIN SAVED/SHIFT	STAFFED BEDS
46,976	5,792	30	143

HOW TO READ IT

Hours returned/yr is the headline – it’s what reclaimed end-of-shift documentation time looks like when aggregated across the unit. Minutes saved/shift is the per-nurse experience that supports adoption. Patient days/yr and staffed beds anchor the math to your actual operating footprint.

We deliberately don’t price capacity. Translating reclaimed minutes into dollars requires assumptions about what the next hour gets used for – assumptions that vary by unit, shift, and patient mix. Tracking the leading indicator is more defensible than monetizing it.

Real-time documentation is the visibility layer that makes early intervention possible.

Preventable harm events don't happen because nurses don't care — they happen when risk signals are missed or delayed. The hypothesis we model: real-time flowsheet capture surfaces those signals while there's still time to act, with the actual outcome determined by clinical practice on each unit.

The values below are potential — they require clinical practice change alongside documentation improvement. Documentation creates the visibility; the care team creates the outcome. Validate baseline rates with your infection-control and quality teams before presenting.

HAPI RISK REDUCTION

\$213K (potential)

Skin assessments captured at the point of care surface risk earlier than charts reconstructed at shift end. The model isolates the documentation-attributable share of preventable HAPIs.

PATIENT DAYS/YR	46,976
HAPI RATE PER 1,000 PATIENT DAYS	2.79
ESTIMATED HAPIS/YR	131.1
DOCUMENTATION-ATTRIBUTABLE PREVENTION	6.5%
COST PER EVENT	\$25,000
= POTENTIAL VALUE	\$213K

Source: Dowding et al., JAMIA 2012.

FALL RISK VISIBILITY

\$98K (potential)

Morse Fall Scale assessments completed in real time can make risk escalations visible when they matter, rather than at shift end.

PATIENT DAYS/YR	46,976
FALLS RATE PER 1,000 PATIENT DAYS	3.2
ESTIMATED FALLS/YR	150.3
DOCUMENTATION-ATTRIBUTABLE PREVENTION	10%
COST PER EVENT	\$6,500
= POTENTIAL VALUE	\$98K

Source: AHRQ inpatient fall cost benchmarks.

Why revenue lives elsewhere in this model.

The nursing ROI story is a quality and workforce story.

In the physician and APP care settings, ambient documentation creates a measurable revenue story – wRVU capture, DRG accuracy, denial prevention. For nursing, the picture is different: nursing documentation primarily informs care decisions and shapes labor outcomes rather than driving the revenue cycle, so the financial impact shows up in quality and workforce instead of billing.

This model focuses on the value drivers where nursing has the strongest, most defensible footprint: the QUALITY bucket (preventable harm events, sepsis bundle compliance) and the WORKFORCE bucket (retention, agency cost avoidance, overtime reduction). Keeping the revenue narrative with the care settings that actually drive billing makes both stories more credible.

THE INDIRECT CONNECTION

Complete nursing documentation does create downstream organizational value: HCAHPS performance affects Value-Based Purchasing scores. HAC scores affect Medicare payment rates. Sepsis SEP-1 compliance affects public reporting and payer relationships. These flow through the model as quality outcomes rather than revenue line items – which keeps the framing both honest and defensible.

Infrastructure, Not Expense.

The investment is \$206K annually – \$120/bed/month at 143 staffed beds. Years 2–3 assume 10% growth as adoption matures and documentation habits stabilize across the unit.

PERIOD	VALUE	INVESTMENT	NET VALUE	CUMULATIVE
Year 1	\$982K	\$206K	\$776K	\$776K
Year 2	\$1.08M	\$206K	\$874K	\$1.65M
Year 3	\$1.19M	\$206K	\$982K	\$2.63M

Under the modeled assumptions, by Year 3 the program projects 4.3× cumulative net for every \$1 invested – alongside more time at the bedside and less end-of-shift charting for nursing staff.

■ YOUR ASSESSMENT SUMMARY

PROJECTED NET ANNUAL VALUE

\$776K

143 staffed beds · 149 nurse FTEs · \$5,427/bed per year

VALUE SUMMARY

QUALITY	\$311K (potential)
HAPI Prevention	\$213K
Falls Prevention	\$98K
CAUTI Prevention	—
CLABSI Prevention	—
Sepsis Bundle Compliance	—
HAC Penalty Exposure	Risk display only
HCAHPS / Patient Experience	—
Medication Error Reduction	—
WORKFORCE	\$671K
RN Retention	\$90K
Agency & Travel Nurse Reduction	\$48K
Overtime Reduction	\$533K
CAPACITY	Tracked
Bedside / Direct Care Time	Tracked
REVENUE	Tracked Separately
Captured in the physician & APP models	—
NET ANNUAL VALUE (HARD)	\$776K

METHODOLOGY

- HAPI: 2.79/1,000 patient days. 6.5% documentation-attributable prevention rate. \$25,000/event. Source: Dowding et al., JAMIA 2012.
- Falls: 3.2/1,000 patient days. 10% documentation-attributable prevention rate. \$6,500/event. Source: AHRQ inpatient fall cost benchmarks.
- Retention: 18% annual turnover, 40% burnout-related. 15% impact scenario. \$56,300 replacement cost. Source: NSI Nursing Solutions 2024.
- Agency: 12 weeks coverage per vacancy at \$2,500/week premium.
- OT Reduction: 2.3 OT hrs/nurse/week at 40% reduction. \$75/hour.
- HAC Penalty: 1% of Medicare revenue if in bottom quartile. Risk display only – not included in ROI total.
- Revenue: Tracked separately in the physician and APP models, where billing originates.

This assessment is for planning purposes. Hard value projections are based on user-provided staffing inputs. Potential value uses published clinical rates with documentation-attributable prevention rates that reflect the indirect causal chain. Validate with your organization's data post-implementation.